ZABARWAN SURGICAL & CANCER HOSPITAL
GI Surgery | Surgical Oncology | Pathology & Laboratory Medicine
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OPERATION RECORD

Field	Entry
Patient	**Ghulam Nabi Dar, 67 Y / M**
IP No. / UHID	ZSH/IP/26/3318 / ZSH-0097215
Date of Operation	**21 Jul 2026**
Time	Incision 09:40, closure 14:05
Surgeon	Dr. Showkat Ahmad Mir
Assistant(s)	Dr. Insha Bashir, Dr. Aamir Hussain Lone
Anaesthesia	GA + epidural (Dr. Nusrat Jan)
Pre-operative diagnosis	Adenocarcinoma of the gastric antrum
Operation performed	**Open radical distal (subtotal) gastrectomy with D2 lymphadenectomy and Roux-en-Y gastrojejunostomy**

Operative findings

A 6 x 5 cm hard growth in the antrum extending to the distal body along the lesser curve; serosa puckered, adhesion to the pancreatic head separated easily without infiltration. Enlarged nodes at stations 3, 4d, 6 and 8a. Liver, peritoneum and pelvis free of disease.

Procedure

Upper midline incision. Peritoneal wash taken. Omentectomy and division of the right gastroepiploic and right gastric vessels at their origins. Duodenum divided 2 cm beyond the pylorus with a linear stapler. D2 nodal clearance (stations 1, 3, 4sb, 4d, 5, 6, 7, 8a, 9, 11p, 12a); left gastric artery ligated at its origin. Stomach transected leaving a 4 cm remnant. Antecolic Roux-en-Y gastrojejunostomy (45 cm Roux limb) and jejunojejunostomy, hand-sewn. Drain in the subhepatic space.

Specimen: stomach with greater omentum and separately labelled nodal stations, sent for histopathological examination. Peritoneal wash sent for cytology.

EBL about 300 mL. Extubated on table and shifted to SICU, haemodynamically stable.

Dr. Showkat Ahmad Mir
M.S., M.Ch. (Surgical Oncology)
Reg. No. JKMC-5523
Consultant Surgical Oncologist

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY